

Demo Clinical Record Package

Fictional patient case designed for testing clinical summary generation, entity extraction, and documentation gap detection.

Patient	Jordan Lee
Date of Birth	03/14/1962
Demo MRN	DEMO-2026-001
Encounter Date	06/18/2026 - 06/21/2026
Facility	Austin Metro Medical Center - Fictional
Document Type	Emergency Department Note, Inpatient Progress Note, Medication List, Discharge Summary
Confidentiality	Synthetic data only; no real patient identifiers or protected health information.

Emergency Department Note

Chief Complaint

Shortness of breath, worsening fatigue, and bilateral lower extremity swelling for three days.

History of Present Illness

Jordan Lee is a 64-year-old patient with a history of type 2 diabetes mellitus, hypertension, chronic kidney disease, and congestive heart failure who presented to the emergency department after three days of progressive dyspnea on exertion, orthopnea, and ankle swelling. The patient reports needing three pillows to sleep and notes a five-pound weight gain over one week. The patient denies chest pain, syncope, fever, or recent travel. Home blood glucose readings have been elevated, typically 180-240 mg/dL.

The ED clinician documented 'CHF exacerbation' and 'possible pneumonia' in the impression. The note does not consistently specify whether the heart failure is acute, chronic, systolic, or diastolic. Chest imaging showed pulmonary vascular congestion, while infection markers were only mildly abnormal.

Past Medical History

- Type 2 diabetes mellitus with hyperglycemia; most recent A1c documented as 8.1% during this admission.
- Hypertension, long-standing, treated with lisinopril and metoprolol as outpatient.
- Congestive heart failure, prior outpatient cardiology follow-up; baseline ejection fraction not available in the ED note.
- Chronic kidney disease; stage is not stated in the admitting note.
- Hyperlipidemia.

Allergies

Penicillin - rash reported in childhood. No documented anaphylaxis.

Vital Signs on Arrival

Time	BP	HR	RR	Temp	SpO2	Weight
06/18/2026 08:42	162/92	104	22	98.6 F	91% on room air	214 lb
06/18/2026 10:15	148/86	96	20	98.4 F	95% on 2 L nasal cannula	Not repeated

Physical Examination

- General: Alert, mildly uncomfortable, speaking in full sentences.
- Cardiovascular: Tachycardic, regular rhythm, no documented murmur.
- Respiratory: Bibasilar crackles; no wheezing; mild increased work of breathing.
- Extremities: 2+ bilateral pitting edema to mid-shin.
- Skin: Small plantar foot ulcer noted; size, depth, drainage, and laterality are not documented.

Diagnostic Data and Hospital Course

Laboratory Results

Test	Result	Reference / Note
BNP	1,240 pg/mL	Elevated; supports volume overload
Creatinine	1.8 mg/dL	Baseline listed as 1.3 mg/dL in prior chart
eGFR	42 mL/min/1.73m2	CKD stage not documented by provider
BUN	36 mg/dL	Elevated
Sodium	133 mmol/L	Mild hyponatremia
Potassium	4.7 mmol/L	Within range
Troponin I	Negative x2	No acute myocardial injury documented
Hemoglobin A1c	8.1%	Diabetes control above goal
WBC	11.2 K/uL	Mild leukocytosis
Procalcitonin	0.09 ng/mL	Low; bacterial pneumonia less likely

Imaging and Tests

- Chest X-ray: Mild pulmonary vascular congestion and small bilateral pleural effusions. No focal lobar consolidation.
- ECG: Sinus tachycardia. No ST elevation or acute ischemic changes.
- Echocardiogram 06/19/2026: Left ventricular ejection fraction estimated at 35-40%; mild global hypokinesis; no significant valvular stenosis. The provider note refers to CHF but does not always connect the reduced ejection fraction to systolic heart failure documentation.
- Telemetry: No sustained arrhythmia documented during admission.

Medication Administration Record

Medication	Dose / Route	Frequency	Indication / Comment
Furosemide	40 mg IV	Twice daily	Volume overload / CHF exacerbation
Metoprolol succinate	50 mg PO	Daily	Heart failure / hypertension
Lisinopril	Held	N/A	Held due to acute kidney injury concern
Insulin glargine	18 units SQ	Nightly	Diabetes mellitus
Insulin lispro	Sliding scale SQ	Before meals	Hyperglycemia correction
Atorvastatin	40 mg PO	Nightly	Hyperlipidemia

Medication	Dose / Route	Frequency	Indication / Comment
Ceftriaxone	1 g IV	Daily	Started for possible pneumonia, but final diagnosis unclear
Albuterol	2 puffs	PRN	Shortness of breath; no asthma/COPD diagnosis documented

Hospital Course Summary

The patient was admitted for suspected acute decompensated heart failure with hypoxia and volume overload. IV diuresis was started with improvement in oxygen saturation and edema. Net negative fluid balance was approximately 2.1 liters by hospital day two. Creatinine increased from a baseline of 1.3 mg/dL to 1.8 mg/dL on admission and improved to 1.6 mg/dL prior to discharge after ACE inhibitor was held and volume status improved.

The patient received empiric ceftriaxone because the ED impression included possible pneumonia. Subsequent imaging and low procalcitonin did not strongly support bacterial pneumonia, and the discharge summary does not clearly state whether pneumonia was ruled out, treated, or remained suspected.

Assessment, Plan, and Discharge Summary

Problem List and Plan

Problem	Assessment / Plan
Heart failure exacerbation	Improved with IV furosemide. Echo EF 35-40%. Continue metoprolol. Restart ACE inhibitor when renal function stabilizes. Daily weights and low-sodium diet advised. Specific acuity and systolic/diastolic type are inconsistently stated across notes.
Acute kidney injury on chronic kidney disease	Creatinine peaked at 1.8 mg/dL and improved to 1.6 mg/dL. CKD is documented, but CKD stage is not explicitly documented despite eGFR 42.
Type 2 diabetes mellitus	A1c 8.1%. Continue basal insulin and resume metformin only if renal function allows. Hyperglycemia documented during admission.
Possible pneumonia	Ceftriaxone started in ED. No focal consolidation on imaging. Final status is unclear: not clearly confirmed, ruled out, or discontinued as suspected diagnosis.
Hypertension	Continue beta blocker. Lisinopril held because of renal function. Blood pressure improved but remained above goal.
Plantar foot ulcer	Small ulcer mentioned on exam. Missing laterality, size, depth, drainage, staging, and wound care follow-up plan.

Discharge Medications

Medication	Instruction
Furosemide	40 mg by mouth daily; increase to twice daily only if instructed by clinician.
Metoprolol succinate	50 mg by mouth daily.
Insulin glargine	18 units nightly.
Insulin lispro	Use sliding scale as directed with meals.
Atorvastatin	40 mg by mouth nightly.
Metformin	Hold until follow-up renal labs are reviewed.
Lisinopril	Hold at discharge; consider restart after kidney function reassessment.

Discharge Instructions

- Follow up with primary care within 7 days for renal function, diabetes management, and blood pressure review.
- Follow up with cardiology within 10-14 days for heart failure management and medication optimization.
- Check daily weight. Call clinician for weight gain greater than 2 lb in 24 hours or greater than 5 lb in one week.
- Follow low-sodium diet and take medications as prescribed.
- Return to emergency care for chest pain, worsening shortness of breath, fainting, fever, or severe swelling.

Documentation Elements Useful for AI Review

This synthetic record intentionally includes several areas that a clinical review assistant should surface for human review: inconsistent heart failure specificity, missing CKD stage, unclear pneumonia status, incomplete wound details, and missing medication restart criteria for held ACE inhibitor therapy. These items are included for demonstration of documentation-gap detection and should not be interpreted as real clinical guidance.